Procedure visit - Mar 27, 2026

with Anand Bery, MD at Mass Eye and Ear Vestibular Lab

Notes from Care Team

Progress Notes by Anand Bery, MD at 3/27/2026 12:00 PM

VESTIBULAR TESTING (Mass Eye and Ear)

Location: MASS EYE AND EAR VESTIBULAR LAB

VIDEO-OCULOGRAPHY (VOG) (3/27/2026)
Testing was ordered and completed due to the complexity of the case, and his complaints of dizziness, vertigo, and imbalance which could have a peripheral (benign) or central (potentially dangerous) vestibular localization. Note that VOG by itself cannot be used to diagnose unilateral or bilateral vestibular loss, nor can it distinguish peripheral from central disorders in the acute vestibular syndrome without the addition of the video head impulse test (vHIT).

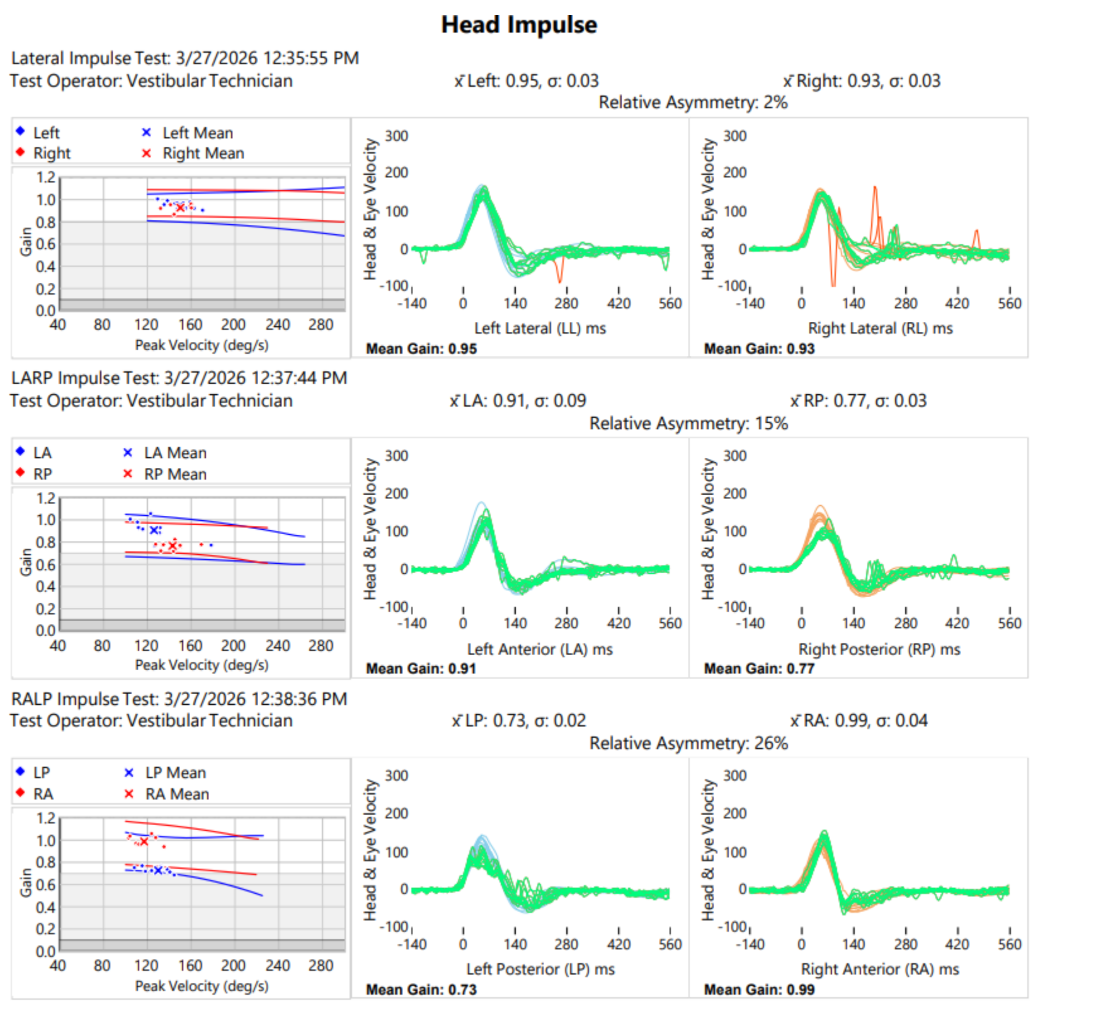
On VOG with fixation, the following were noted:
There was no spontaneous nystagmus
Pursuit: normal
OKN: normal

With video-oculography (VOG - monocular OD) and removal of visual fixation, there was no spontaneous nystagmus. Provocative maneuvers demonstrated the following:
There was head-shaking-induced mild LB nystagmus
There was no bow-induced nystagmus
There was no lean-induced nystagmus

There was left Dix-Hallpike-induced mild LB nystagmus with minimal symptoms, no reversal on sitting up
There was right Dix-Hallpike-induced very mild RB nystagmus

Interpretation: Mild asymptomatic nonspecific positional nystagmus

VIDEO HEAD IMPULSE TESTING (vHIT) (03/27/2026): Testing was ordered and completed due to the complexity of the case, and his complaints of dizziness, vertigo, and imbalance which could have a peripheral (benign) or central (potentially dangerous) vestibular localization. Note that vHIT is essential to distinguish peripheral from central disorders in the acute vestibular syndrome, and for the diagnosis of unilateral or bilateral vestibular loss acutely and chronically - these diagnoses cannot be made with video-oculography alone. vHIT provides a unique measurement of the function of each of the six semicircular canals during head motions that are similar to those encountered in normal daily life. There are no other vestibular tests that provide this same information.



In the plane of the horizontal canals:
Gain left 0.95; Gain right 0.93; Relative asymmetry 2%

vHIT interpretation: normal without clear indication of a peripheral vestibular disorder

Final interpretation: Overall normal study

Anand Kumar Bery, MD
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Mass Eye and Ear